

Patient : J.M | D.O.B : 03/14/1962

Date : 03/04/2026.

Provider : Dr xxx | location : xxx

Chief complaint : Follow up for hypertension and type 2 diabetes mellitus

History of presenting illness:

J.M is a 65 year old male with a past medical history of hypertension , hyperlipidemia and type 2 diabetes , presenting as a new patient for establishment of care . He reports a home blood pressure range of 150-170/ 90-100 over the past 3 months . He attributes this to running out of his lisinopril about 6 weeks ago . patient endorses fatigue occurring daily , worse on exertion and denies chest pain, shortness of breath, palpitations or syncope . patient reports nocturia and increased thirst over the past month . Last A1C was 8.2%, done 8 months ago . He is currently not on any diabetic medication.

Surgical history : Appendectomy

Family history : Father had MI at age 58, Mother has a history of T2DM.

Social history : Former smoker with 15 year pack history , takes alcohol occasionally , denies illicit drug use , works as a warehouse manager, married and lives with spouse

Medications : Lisinopril 10mg daily (lapsed x 6 weeks)

Atorvastatin 20mg daily

Review of system

REVIEW OF SYSTEMS Pertinent positives are addressed in the HPI

Constitutional : denies fever /chills. Weight loss

CV: denies chest pain, palpitations, edema

Respiratory : denies dyspnea, cough\

GI : denies nausea, vomiting , abdominal pain

Neuro : denies headache , dizziness , vision changes

Physical examination

Physical Exam (Telehealth – Constitutional only) Appears in no acute distress; alert and oriented with appropriate affect. Speaking full sentences without visible respiratory distress. No visible rash. Physical examination is limited due to the nature of a telehealth encounter(or if physical I document as appropriate ?

Assessment and plan

Hypertension :
uncontrolled

-Restart lisinopril 10mg daily. Increase to 20mg at follow-up if BP remains >140/90. Counseled on dietary sodium restriction and DASH diet. Repeat BP check in 4 weeks.

-Monitor for s/s of headaches, blurred vision, dizziness, edema, chest pain, dyspnea, palpitations -Follow low sodium/heart healthy diet -Goal BP less than 140/90 -Continue to monitor BP daily

Type 2 Diabetes Mellitus
uncontrolled

-A1c ordered today.

-Start metformin 500mg BID with meals given no contraindications.

-Check feet daily for any wounds or ulcerations

-Nurse/provider to perform toenail care and/or trimming per facility protocol

-Obtain annual diabetic eye exam

-Check A1c per protocol

-Follow up as scheduled

Hyperlipidemia

-Continue current medications as prescribed (Tabs Atorvastatin 20mg daily)

-Eat heart-healthy/low-cholesterol diet

-Increase physical activity as tolerated

-Check lipid panel per protocol

Health Maintenance

Colonoscopy due (last unknown, patient to confirm). Flu vaccine administered today. Low-dose aspirin discussed, deferred pending cardiovascular risk stratification.

Labs ordered: CMP, CBC, A1c, lipid panel, TSH, urine microalbumin

Follow-up: 4 weeks